

ADRABETADEX Document B

Provider net cost model concept brief and demo calculator narrative

Prepared for client discussion. Content is grounded in the supplied source pack and current CMS, HRSA, Medicaid, Medicare, and public payer materials reviewed on March 30, 2026.

What the provider model measures

A provider net cost model for ADRABETADEX measures whether a center of excellence can recover drug acquisition cost, administration cost, and working-capital exposure on a denial-adjusted basis. This is the correct operational question for launch because buy-and-bill transfers inventory, coding, payment, and timing risk to the provider.

The model in this document uses the formula from the source pack: **Net Cost Recovery per administration = expected drug revenue + expected administration revenue - drug acquisition - non-reimbursed drug loss - administration cost - denial/rework cost - cost of capital on receivables.**

Data stack used in the demo

- Drug acquisition anchor: \$39,000 per 900 mg vial every two weeks from the source pack, or \$1,014,000 per patient-year before overhead.
- Drug payment benchmark: \$40,170 per administration based on the CMS launch-period benchmark of 103% of WAC before ASP is available.
- Administration revenue: fixed demo values of \$1,850 for commercial / 340B HOPD and \$1,500 for Medicaid HOPD to reflect procedure-related reimbursement in a specialized outpatient setting.
- Operational frictions: non-reimbursed drug loss, denial/rework cost, and cost of capital based on 45-day to 60-day payment lags.
- 340B module: new-drug 340B acquisition estimated from WAC minus 23.1% until AMP is available.

Demo results

Scenario	Acquisition cost	Expected total revenue	Net margin / administration	Annualized net margin	Peak drug float
Commercial medical benefit, non-340B HOPD	\$39,000	\$41,180	\$-145	\$-3,771	\$156,000
Medicaid pediatric, non-340B HOPD	\$39,000	\$39,587	\$-2,661	\$-69,196	\$195,000
340B pediatric COE, HOPD	\$29,991	\$41,180	\$9,043	\$235,115	\$119,964

Interpretation

- Commercial medical-benefit economics are nearly break-even in the demo even before white-bagging pressure is layered in. A center can remain clinically interested in ADRABETADEX and still be reluctant to scale it because the dollar spread is thin.

- The Medicaid non-340B case is negative because pediatric coverage does not remove documentation friction, slower payment, or higher rework burden.
- The 340B center-of-excellence case is strongly positive, which means 340B status changes behavior. The model therefore has to split 340B and non-340B sites instead of averaging them.
- A biweekly regimen plus a 45-day to 60-day lag creates a visible float problem. The demo range is **\$156,000 to \$195,000** of drug inventory tied up before payment arrives at non-340B sites.

Why the model matters for launch execution

Expanded access experience does not answer the launch economics question because expanded access begins with manufacturer-supplied investigational product. Once ADRABETADEX converts to commercial supply, the provider finances acquisition and bears the billing cycle.

Commercial plans already document the tools that compress provider margin. Public UHC materials describe specialty-pharmacy sourcing protocols that can prevent providers from billing for the drug itself when the payer routes the product through a designated pharmacy. That moves ADRABETADEX from buy-and-bill economics to administration-only economics at the affected sites.

The model therefore belongs in launch governance for three reasons: it converts reimbursement into provider cash flow, it highlights the extra value of 340B and clean Medicaid operations, and it identifies where coding gaps can suppress early adoption even when coverage exists on paper.

Inputs that need owner-level governance

- Dose and vial assumptions by patient weight band, including partial-vial loss policy.
- Allowed-amount logic by payer archetype and site of care.
- Denial, appeal, and resubmission assumptions tied to interim coding.
- Payment-lag assumptions by channel, with separate views for Medicaid FFS and Medicaid managed care.
- 340B carve-in / carve-out treatment and duplicate-discount controls at target centers.

References

Source pack: Project Castar internal commercialization deck supplied in the source pack.

Source pack: Provider Net Cost Model Concept Brief and Launch Access Reality Addendum for ADRABETADEX supplied in the source pack.

Source pack: ADRABETADEX U.S. Reimbursement & Policy Deep Research Report supplied in the source pack.

UnitedHealthcare Miplyffa prior authorization criteria, effective April 1, 2025.

<https://www.uhcprovider.com/content/dam/provider/docs/public/prior-auth/drugs-pharmacy/commercial/h-p/PA-Notification-Miplyffa.pdf>

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<https://www.uhcprovider.com/content/dam/provider/docs/public/prior-auth/drugs-pharmacy/commercial/a-g/PA-Notification-Aqneursa.pdf>

HRSA 340B pricing formula page.

<https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/Pricing%20Formulas.htm>

HRSA new drug price estimation page.

<https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/New%20Drug%20Price%20Estimation.htm>

Medicaid.gov EPSDT page. <https://www.medicaid.gov/medicaid/benefits/early-and-periodic-screening-diagnostic-and-treatment>

Medicaid.gov PAD page.

<https://www.medicaid.gov/medicaid/prescription-drugs/state-prescription-drug-resources/physician-administered-drugs-pad>

Medicare and Medicaid quick guide, July 2024. <https://www.medicare.gov/publications/12207-a-quick-guide-medicare-medicaid.pdf>

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CMS OPPS pass-through and new technology APC application page. <https://www.cms.gov/medicare/payment/prospective-payment-systems/hospital-outpatient/pass-through-payment-status-new-technology-ambulatory-payment-classification-apc>

CMS OPPS drug and biologic transitional pass-through instructions, updated March 16, 2026.

<https://www.cms.gov/files/document/determine-eligibility-drugs-biologicals-transitional-pass-through-payment-under-hospital-outpatient.pdf>

CMS HCPCS Level II coding decisions page.

<https://www.cms.gov/medicare/coding-billing/healthcare-common-procedure-system/current-prior-years-level-II-coding-decisions>

CMS 2025 first-quarter HCPCS drug and biologic coding determinations.

<https://www.cms.gov/files/document/2025-hcpcs-application-summary-quarter-1-2025-drugs-and-biologicals.pdf>